

THE AESTHETICS PILOT / MEDLYFE AESTHETICS

Execution Plan and Financials

Month by month, the gaps to close, and what it takes.

PREPARED FOR

Dr Chinwe Kpaduwa (KP Plastics) and Dr Itunu Akinware (Medbury Healthcare Group)

BY

Consult for Africa

HORIZON

Next 3 months (July to September), decision in November

STATUS

Draft for discussion · benchmark figures for tuning



01 / THE PLAN, MONTH BY MONTH

From agreement to a living practice.

The companion to the operating model, focused on sequence, gaps and numbers. Each month has a clear purpose and a visible milestone, so the decision in November rests on what has actually been built.

Period	Focus	Milestone
To end July	Conclude the vision, the contracts and the paperwork, including the JV establishment and the formal registrations	Agreement, structure and registrations in place
August	Recruit the team, from within Medbury and externally; Dr Kpaduwa leads the practice design, protocols and virtual consultations, supported by C4A; the C4A and Medbury teams drive marketing and inbound enquiries	Team, protocols and demand taking shape
September	The practice goes live: the team delivers to Dr Kpaduwa's standard; the first patients and the first revenue	Live and delivering
Into November	The practice grows; review the demand, the economics and the model; decide together on the standalone	Go or no-go on the standalone

The next 15 days: conclude the deal

- Finalise and sign the collaboration heads of terms.
- Establish the JV and begin the formal facility registrations.
- Confirm the venue, agree the menu pricing and the pilot revenue split.
- Begin early procurement of the long-lead and imported equipment, to Dr Kpaduwa's specification.
- Agree Dr Kpaduwa's licensing pathway and the partner-theatre arrangement for surgical cases.

02 / THE KEY GAPS TO CLOSE

What stands between us and launch.

Named honestly, with an owner and a moment each is closed.

Gap	What it is	When it closes
The financials	Sized in this document; to firm on the equipment list and the venue.	Closed here, to firm
JV and registrations	Establish the JV, and the formal facility registrations.	July
Licensing and privileges	Dr Kpaduwa's Nigerian practice licence, and partner-theatre operating privileges for surgical cases.	July onward
Equipment procurement	Long-lead and imported items to Dr Kpaduwa's specification, ordered early.	Starts now
Pricing and revenue split	The menu pricing, and the pilot revenue split between the partners.	By end July
The team	Recruit the resident doctor and nurse or nurses, from within Medbury and externally.	August

03 / SETUP CAPITAL AND WORKING CAPITAL

What it takes to open.



The pilot uses the existing Medlyfe clinic, so there is no build. The money is a modest setup spend, fit-out and the equipment to Dr Kpaduwa's specification, plus a working-capital float to carry the pre-revenue months before the practice goes live in September.

Setup capital (one-off)

Item	NGN M
Clinic fit-out and finishing	5
Equipment to Dr Kpaduwa's specification (clinical, aesthetic, diagnostic, sterilisation, instrument set)	12
HospitlOS, IT, branding and pre-opening	4
Build subtotal	21
Contingency (20%)	4
Total setup capital	25

Equipment is the swing, procured to Dr Kpaduwa's specification; laser and heavier devices are added later from cashflow, so capital follows demand. Some items are imported, so procurement starts now to protect the September launch.

Working capital (float through the pre-revenue build)

Item	NGN M
Operating float through July and August, before revenue	12
Opening consumables and inventory	3
Total working capital	15

Total pilot funding requirement: about NGN 40M (setup capital plus working capital). Because the practice goes live in September, most of the spend comes before the first revenue; peak cash out is about NGN 35M by end September, recovering through the fourth quarter as the diary builds.

04 / THE FINANCIALS, MONTH BY MONTH

In cash, honestly.

The clinic-level cash view. July and August are investment, the deal, the setup and the marketing build; revenue begins in September when the practice opens. Figures are benchmarks, to firm on the pricing and the warm-base response.

NGN M	July	August	September	3-month
Revenue	0	0	8	8
Cost of care (about 30%)	0	0	(2.4)	(2.4)
Staff and clinic team	(1.0)	(2.5)	(2.5)	(6.0)
Marketing	(1.0)	(3.0)	(3.0)	(7.0)
Facility and admin	(0.5)	(1.0)	(1.0)	(2.5)
Operating cash flow	(2.5)	(6.5)	(0.9)	(9.9)
Setup capex	(15)	(10)	-	(25)
Net cash flow	(17.5)	(16.5)	(0.9)	(34.9)



Cumulative cash (17.5) (34.0) (34.9)

- **The operating cash flow is before the revenue split.** The margin is shared among the partners per the pilot split, to be agreed; Medbury funds the setup and the float.
- **Revenue begins in September** and builds through the fourth quarter; the September figure is the opening month of the launch, into the warm base.
- **This is an investment window, not a profit engine.** The point is a live, credible practice and the evidence for the November decision, at the lowest sensible cost.

05 / GOVERNANCE AND THE GATE

How we stay on track, and decide.

- **Weekly** operations review on setup progress, then bookings and delivery once live; **monthly** a principals' review against the plan and the numbers.
- **In November** the partners review the practice, the demand, the economics and the model, and decide together on the standalone.

The November decision

Outcome	What happens
If it works	Form the standalone SPV, a dedicated centre with the cosmetic surgery arm, built on the team and the standard the pilot has created; the fuller structure is settled on the pilot's real numbers.
If not yet	Medlyfe continues the aesthetics service with the trained team, and the relationship stays open. Nothing is stranded.

Consult for Africa Structuring and operating partner
hello@consultforafrica.com / +234 913 813 8553 / consultforafrica.com

Draft execution plan and financials shared in confidence with Dr Chinwe Kpaduwa and Dr Itunu Akinware. Capital, working capital and revenue are benchmark estimates, to firm on the equipment specification, the venue and the agreed pricing. The strategy sits in the companion target operating model. Prepared by Consult for Africa.